

Title

Planned Early Delivery in Treated Hypertensive Pregnancy: Maternal and Neonatal Outcomes from the TERM-HYP Trial

Abstract

Four hundred three pregnant women with hypertensive disorders at term were randomized to either planned delivery at 38 weeks (n = 201) or standard management (n = 202). Baseline characteristics were balanced; most participants had well-controlled blood pressure on antihypertensive therapy. The planned early delivery group delivered a median of 0.9 weeks earlier (38.4 versus 39.3 weeks gestation). Severe maternal complications including hypertensive emergency, maternal death, or ICU admission occurred in 13% of the early delivery group and 12% of the standard management group (adjusted risk ratio 1.14, 95% CI 0.70-1.86). Neonatal intensive care admission for 4 or more hours occurred in 7% of both groups (adjusted risk difference -0.01, 95% CI -0.06 to +0.07), meeting non-inferiority criteria. Cesarean delivery rates were 29% versus 36% (adjusted risk ratio 0.79, 95% CI 0.59-1.06). No serious adverse events were reported. Planned delivery at 38 weeks offers a safe alternative in hypertensive pregnancy.